

PACKAGE INSERT



FOLIC ACID TABLETS 5MG

Each tablet contains:-
Folic Acid 5 mg

Product Description: Round, pale yellow, standard convex, plain tablet with score on one side.

Pharmacodynamics:

Folic acid is a member of the vitamin B. Folic acid is reduced in the body to tetrahydrofolate, which is a coenzyme for various metabolic processes including the synthesis of purine and pyrimidine nucleotides and hence in the synthesis of DNA; it is also involved in some amino-acid conversions and in the formation and utilisation of formate.

Pharmacokinetics:

Folic acid is absorbed in the proximal small intestine, particularly the duodenum. Folic acid is absorbed maximally and rapidly at this site. After absorption folic acid is rapidly converted into its metabolically active forms. Approximately two-thirds is bound to plasma protein. Half of the folic acid stored in the body is found in the liver. Folic acid is also concentrated in spinal fluid. Except for the folates ingested in the liver, yeast and egg yolk, the percentage of absorption of food folates averages about 10%.

Indication:

It is indicated for the treatment of megaloblastic anaemia caused by deficiency of folic acid. It is also indicated in pregnancy to supply a maintenance dosage of folic acid.

Recommended Dosage:

In the treatment of megaloblastic anaemia, folic acid may be given in an initial dosage of 10 to 20mg daily for 14 days, or until a haemopoietic response is obtained. The daily maintenance dose is 2.5 to 10mg. Children may be given 5 to 15 mg daily according to the severity of the deficiency state.

Route of Administration:

Oral

Contraindications:

It is contraindicated in patients who are sensitive to it.

Warnings and Precautions:

Folic acid should never be given alone or in conjunction with inadequate amounts of hydroxocobalamin for the treatment of megaloblastic or pernicious anaemia. Though folic acid produces a haemopoietic response, it fails to prevent the onset of subacute combined

degeneration of the cord. Folic acid should not be given before a diagnosis has been fully established. Large and continuous doses of folic acid may lower the blood concentration of vitamin B12. The availability of folates may be inhibited by gastric acidity and the presence of ascorbate in the gastrointestinal tract. Metformin reduces intestinal absorption of folic acid and phenytoin may cause folate depletion. Folic acid does not correct folate deficiency due to dihydrofolate reductase inhibitors.

Interactions with other Medicaments:

Folate status may be affected by a number of drugs and anticonvulsants, oral contraceptives, antituberculous drugs, alcohol and folic acid antagonists including aminopterin, methotrexate, pyrimethamine, trimethoprim and sulphonamides have all been said to produce folate deficiency states.

Pregnancy and Lactation:

To supply maintenance dosage of folic acid to pregnant and lactating woman.

Side Effects:

Folic acid is usually well tolerated. However, allergic sensitization has been reported following oral administration.

Symptoms and Treatment of Overdose:

The possibility of serious toxic effects due to overdosage appears remote.

Treatment: An emetic should be administered as soon as possible following by gastric lavage if possible.

Storage Conditions:

Store below 30°C.

Pack Size:

Bottle pack: A bottle of 30 and 90 tablets.
Blister pack: A box of 100 x 10 tablets per strip.
A box of 50 x 10 tablets per strip.
A box of 10 x 10 tablets per strip.

Pack Size (export only):

A bottle of 1000 tablets.

Shelf-life:

5 years.

FURTHER INFORMATION CONCERNING THIS DRUG CAN BE OBTAINED FROM YOUR FAMILY PHYSICIAN/LOCAL GENERAL PRACTITIONER/PHARMACIST.

Manufacturer & Product Registration Holder:

SUNWARD PHARMACEUTICAL SDN. BHD.

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